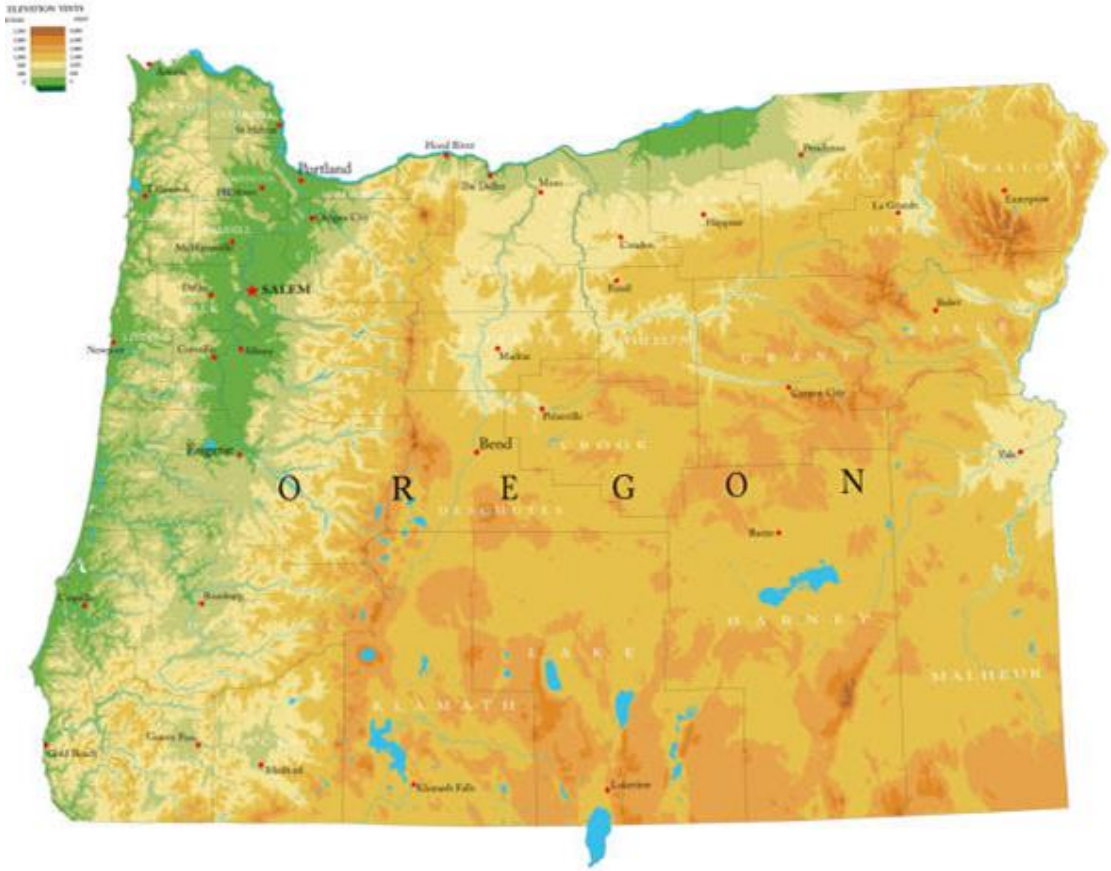


COMBATING ADOLESCENT SUBSTANCE ABUSE IN OREGON

A Community-Centric Approach

By

Navin Kumar Balagopal

BRIEF DESCRIPTION OF INTERVENTION

- What is the issue you are hoping to address?
- What would success look like?
- Who is the target audience of this intervention?
- Who are the people or organizations in charge of implementing this intervention?

INTERVENTIONS & THEORY OF CHANGE

- **Issue Addressed:**
The primary issue aimed at addressing through this intervention is the high rate of substance abuse among adolescents in Oregon, which mirrors a broader national trend. Substance abuse in this age group can lead to a multitude of problems including poor academic performance, mental health issues, and a potential lifetime of addiction.

- **Vision of Success:**
Success would manifest as a significant reduction in the rates of substance abuse among adolescents within the community. This would be measured through a decrease in the percentage of teens reporting drug and alcohol use, improved academic performances, better mental health statistics, and a reduced rate of substance-related incidents within the community.

- **Target Audience:**
The core target audience for this intervention is adolescents aged 12 to 20 residing in Oregon, with a particular focus on those in lower socioeconomic statuses as they often have less access to preventative resources and treatment.

- **Implementing Bodies:**
The intervention will be spearheaded by a coalition comprising the Oregon Health Authority, local schools, community organizations specializing in substance abuse prevention, and local healthcare providers. Engaging a diverse group of stakeholders will ensure a holistic approach to addressing adolescent substance abuse in Oregon, while also fostering community buy-in and sustainability.

ISSUE ADDRESSED:

The intervention primarily targets the escalating rates of substance abuse among adolescents in Oregon.

This problem is reflective of a larger national issue, indicating a need for urgent action.

Consequences of substance abuse in adolescents:

1. Deterioration in academic performance
2. Emergence or exacerbation of mental health issues
3. Potential onset of lifelong addiction issues

Oregon

Teenagers in Oregon are 37.62% more likely to have used drugs in the last month than the average American teen.

- 34,000 or 11.46% of 12- to 17-year-olds report using drugs in the last month.
- Among them, 94.12% report using marijuana in the last month.
- 18.54% of all 12- to 17-year-olds report using marijuana in the last year.
- 0.67% report using cocaine in the last year.
- 0.34% report using methamphetamines.
- Up to 0.17% used heroin (data is limited).
- 3.03% report misusing pain relievers.
- 10.79% of all 12- to 17-year-olds used alcohol in the last month.
- They're 17.91% more likely to use alcohol than the average American in their age group.
- 196,000 adults aged 18- to 25-year-old used drugs in the last month.
- 18- to 25-year-olds in Oregon are 25.51% more likely to use drugs than the average American in the sa age group.
- 4.38% of all 12- to 17-year-olds met the criteria for IDUD in the last year.
- 2.36% of all 12- to 17-year-olds met the criteria for AUD in the last year.

VISION OF SUCCESS

A substantial reduction in adolescent substance abuse rates within the community.

Measurable Outcomes:

Decrease in the percentage of adolescents reporting drug and alcohol use.	Improvement in academic performance indicators among the target age group.	Better mental health statistics among adolescents.	Reduction in substance-related incidents reported within the community, like DUIs, substance-related accidents, etc.

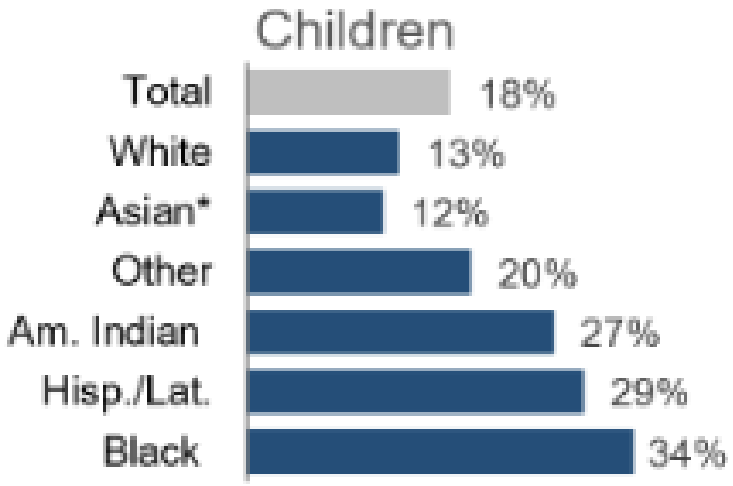
TARGET AUDIENCE:

Adolescents aged 12 to 18 residing in Oregon.

Special emphasis on those belonging to lower socioeconomic statuses due to:

Limited access to preventative resources	Reduced availability of treatment facilities

Poverty Rate Among Children by race in Oregon in 2019



IMPLEMENTING BODIES:

Holistic approach: Diverse stakeholders bring varied expertise ensuring a comprehensive approach to tackling adolescent substance abuse.

Community Buy-in: Involvement of local bodies fosters community acceptance and engagement.

Sustainability: Engaging multiple stakeholders including local authorities ensures a sustained effort towards combating substance abuse, beyond the duration of the project.

SO THAT DIAGRAM

Start Implement comprehensive substance abuse prevention and education programs in schools and communities in Oregon. So that…	Adolescents and their families are well-informed about the risks and consequences of substance abuse. So that…	There's a change in attitude towards substance use and an increased utilization of available substance abuse prevention and treatment resources So that…	Adolescents engage in healthier behaviors, reducing the rates of substance abuse. So that…	End Here (Goal): The community sees improved adolescent mental and physical health, better academic performance, and a safer, more supportive environment for its youth.

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Inputs	Activities	Outputs	Outcomes	Impact
Funding from various sources	Establishing referral processes	Adolescents receiving short-term therapy	Better access to substance abuse prevention and treatment resources	Improved mental and physical health
Trained personnel	Conducting substance abuse education and prevention programs	Families receiving assistance	Improved academic performance	Reduced rates of substance abuse
Local support	Providing short-term therapy and counselling services	Enhanced coordination between local entities	Enhanced community safety	Enhanced community support systems
Substance abuse prevention and treatment resources	Coordinating care and support services	Increased awareness and understanding of substance abuse risks	Improved social relationships among adolescents	A safer and more supportive community environment

LOGIC MODEL DIAGRAM

DATA INDICATORS

	Data Indicators	Items
Input	Amount of funding secured for the program #of Therapists on staff # of community partnerships	Funding Amount Number of therapists Number of partnerships formed
Activities	# of education sessions conducted # of therapy sessions provided # of referrals made to local mental health providers	Substance abuse education sessions Short-term therapy provision Referral process establishment
Outputs	# of adolescents reached # of clients who received short-term therapy # of families assisted	Adolescents receiving education Adolescents receiving short-term therapy Families receiving assistance
Outcomes	% of families accessing long-term services % of families reporting improved access to care % of families satisfied with services received	Long-term services utilization Improved access to care Satisfaction with services

MEASURING POPULATION LEVEL DATA

- 1. Select a data source to help your measure the population level impact of your intervention
- 2. Select the specific indicator you will use to evaluate impact (i.e. % of adults reporting good
- mental health, % of adolescents who report positive relationships)
- 3. Answer the following reflection questions:
- ● How difficult or easy was it to find data that was appropriate for your indicator?
- ● What are some of the limitations of this data? (is it only available for certain people, at
- certain frequencies?)
- ● In an ideal world, what would your ideal data indicator be?
- ● Explain why this particular indicator would be critical evidence to advancing public health
- knowledge of this area

SELECT A DATA SOURCE TO HELP YOUR MEASURE THE POPULATION LEVEL IMPACT OF YOUR INTERVENTION

Data Source: Oregon Healthy Teens Survey (OHT) (2019)

Table 3. Grade demographics

	State 2015				State 2017				State 2019			
	8th		11th		8th		11th		8th		11th	
	%	n	%	n	%	n	%	n	%	n	%	n
Students surveyed	51.5	16,211	48.5	13,608	50.5	14,852	49.5	11,895	51.1	15,256	48.9	12,83

Table 9. Gender Identity and Sex at Birth (State data, 2019)

	What was your sex at birth?											
	Female				Male				Intersex*			
	Grade 8		Grade 11		Grade 8		Grade 11		Grade 8		Grade 11	
How do you identify?	%	n	%	n	%	n	%	n	%	n	%	n
Female	93.6	7,148	94.3	6,121	0.3	23	0.4	17	9.7	3	12.0	2
Male	0.3	24	0.3	15	94.1	7,022	94.4	5,934	12.9	7	4.3	1
Non-binary gender	6.1	476	5.4	320	5.6	396	5.2	329	77.4	40	83.7	25

Table 4. Age demographics

	State 2015				State 2017				State 2019			
	8th		11th		8th		11th		8th		11th	
	%	n	%	n	%	n	%	n	%	n	%	n
12 years old or younger	0.1	27	0.0	0	0.2	31	0.0	0	0.2	24	0.0	0
13 years old	39.4	6,261	0.0	0	36.5	5,450	0.0	0	37.5	5,716	0.0	0
14 years old	58.4	9,485	0.0	0	61.5	8,957	0.0	0	60.3	9,134	0.0	0
15 years old	2.1	339	0.2	34	1.8	290	0.2	22	2.0	276	0.3	26
16 years old	0.0	0	41.1	5,451	0.0	0	35.9	4,292	0.0	0	36.5	4,684
17 years old	0.0	0	56.7	7,788	0.0	0	62.1	7,300	0.0	0	61.2	7,749
18 years old or older	0.0	0	2.0	271	0.0	0	1.8	228	0.0	0	2.0	244

SELECT THE SPECIFIC INDICATOR YOU WILL USE TO EVALUATE IMPACT

(I.E. % OF ADULTS REPORTING GOOD MENTAL HEALTH, % OF ADOLESCENTS WHO REPORT POSITIVE RELATIONSHIPS)

- Percentage of adolescents who report a decrease in substance use.

ANSWER THE FOLLOWING REFLECTION QUESTIONS:

- HOW DIFFICULT OR EASY WAS IT TO FIND DATA THAT WAS APPROPRIATE FOR YOUR INDICATOR?
- WHAT ARE SOME OF THE LIMITATIONS OF THIS DATA? (IS IT ONLY AVAILABLE FOR CERTAIN PEOPLE, AT CERTAIN FREQUENCIES?)
- IN AN IDEAL WORLD, WHAT WOULD YOUR IDEAL DATA INDICATOR BE?
- EXPLAIN WHY THIS PARTICULAR INDICATOR WOULD BE CRITICAL EVIDENCE TO ADVANCING PUBLIC HEALTH KNOWLEDGE OF THIS AREA.

• HOW DIFFICULT OR EASY WAS IT TO FIND DATA THAT WAS APPROPRIATE FOR YOUR INDICATOR?

•

- It was moderately easy to find data appropriate for my indicator as the Oregon Healthy Teens Survey is a well-documented and accessible data source. However, the specificity of substance abuse data particularly changes over time due to interventions, might require additional data-gathering efforts.

● WHAT ARE SOME OF THE LIMITATIONS OF THIS DATA?

(IS IT ONLY AVAILABLE FOR CERTAIN PEOPLE, AT CERTAIN FREQUENCIES?)

- Limitations of the Data:

  The data is collected biennially, which may not provide the timeliest information for evaluating the impact of an intervention.

  It was Last collected in 2019 and since then has been replaced by Oregon Student Health Survey which is less granular and collected biannually. Though this might just be the initial summary available and might be available in the special request data section of the website.

  The survey is school-based, hence, it might miss out on adolescents who are not attending school.

  The self-reported nature of the data might introduce bias.

Topic	Question Wording
Grade	In what grade are you?
Age	How old are you?
Race and ethnicity	What is your race or ethnicity (Select one or more response)?
	If you selected more than one race, what one race best describes you?
Tribal affiliation	Are you enrolled in any of the following tribes?
Language	What is the language you use most often at home?
Youth with disabilities	Are you deaf or do you have serious difficulty hearing?
	Are you blind or do you have serious difficulty seeing, even when wearing glasses?
	Because of a physical, mental or emotional condition, do you have serious difficulty concentrating, remembering or making decisions?
	Do you have serious difficulty walking or climbing stairs?
	Do you have serious difficulty dressing or bathing?
	Because of a physical, mental, or emotional condition, do you have difficulty doing errands alone such as visiting a physician's office or shopping?
Sexual orientation, gender identity and sex assignment	Do you think of yourself as… *Lesbian or gay, Straight or heterosexual, Bisexual, Something else, Don't know/not sure.*
	How do you identify? (Select one or more responses). *Female, Male, Transgender/Trans Female, Transgender/Trans Male, Gender nonconforming, Gender fluid/Genderqueer, Agender, Something else fits better, I am not sure of my gender identity, I do not know what this question is asking.*
	What was your sex at birth? *Female, Male, Intersex and/or my sex was unclear at birth.*

IN AN IDEAL WORLD, WHAT WOULD YOUR IDEAL DATA INDICATOR BE?

Area of Focus	Description
Real-Time Data Collection	- Continuous monitoring and data collection on adolescent substance use.
	- Real-time evaluations of interventions.
	- Prompt identification and response to emerging trends or escalating issues.
Diverse Data Sources	- Data collection from various settings: community centers, healthcare facilities, online platforms, family and social networks.
	- Collaborations with diverse stakeholders for comprehensive data collection.
Comprehensive Data Points	- Understanding various determinants of substance use: socioeconomic factors, peer influence, mental health status, access to substances, education, and awareness levels.
	- Evaluation of intervention effectiveness through comparison of substance use rates pre and post-intervention, evaluating the reach and impact of education and prevention programs, and monitoring the utilization and effectiveness of treatment resources.
Enhanced Evaluative Capabilities	- More precise evaluations of interventions enabling adjustments and improvements based on real-time feedback.
	- Timely evaluations allowing for quicker iterations and improvements to enhance intervention effectiveness.
Holistic Understanding	- Obtaining a well-rounded understanding of adolescent substance use encompassing a wide range of influencing factors.
	- Providing a more comprehensive picture to facilitate better-informed decisions and tailored interventions to address the root causes of substance use among adolescents.

● EXPLAIN WHY THIS PARTICULAR INDICATOR WOULD BE CRITICAL EVIDENCE TO ADVANCING PUBLIC HEALTH KNOWLEDGE OF THIS AREA.

The chosen indicator, reduced substance use among adolescents, is instrumental in gauging the success of the intervention and advancing public health knowledge in this domain

Aspect	Description
Direct Measurement	Directly measures the core objective: reducing adolescent substance abuse.
Real-world Applicability	Provides practical insights on intervention effectiveness in real-world settings.
Evidence-Based Decision Making	Informs better strategy refinement and resource allocation.
Contribution to Literature	Adds to the body of literature on combating adolescent substance abuse.
Holistic Understanding	Offers insights into the broader impact of reducing substance abuse.
Community and Policy Impact	Influences community support and policy frameworks favorably.
Long-term Public Health Benefits	Sets a precedent for tackling similar public health issues, promoting healthier communities.

CITATIONS

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